Chapter 12

Clinical Guidelines for Decision-Making

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Learning Objectives:

Objective 12.1 Overview the rationale for the use of clinical guidelines.
Objective 12.2 Set the framework for the development of a clinical guideline.
Objective 12.3 Introduce the methodology of clinical guideline development.
Objective 12.4 Explain the process of implementation and update of a clinical guideline.

OBJECTIVE 12.1. OVERVIEW THE RATIONALE FOR THE USE OF CLINICAL GUIDELINES

The National Academy of Medicine previously known as Institute of Medicine defines clinical practice guidelines as "*statements that include recommendations, intended to optimize patient care, that are informed by a systematic review of evidence and an assessment of the benefits and harms of alternative care options*."[1] Clinical guidelines recommend how healthcare professionals should care for people with specific conditions. The guidelines can cover any aspect of a condition and may include recommendations for: providing information and advice, prevention, diagnosis, treatment, and longer-term management.[2]

NICE guidelines make evidence-based recommendations on a wide range of topics, from preventing and managing specific conditions, improving health, and managing medicines in different settings, to providing social care and support to adults and children, and planning broader services and interventions to improve the health of communities.

Guideline recommendations set out:

- the care and services that are suitable for most people with a specific condition or need
- the care and services suitable for particular populations, groups or people in particular circumstances or settings
- ways to promote and protect good health or prevent ill health
- the configuration and provision of health and social care services, and/or
- how national and local public sector organisations and partnerships can improve the quality of care and services

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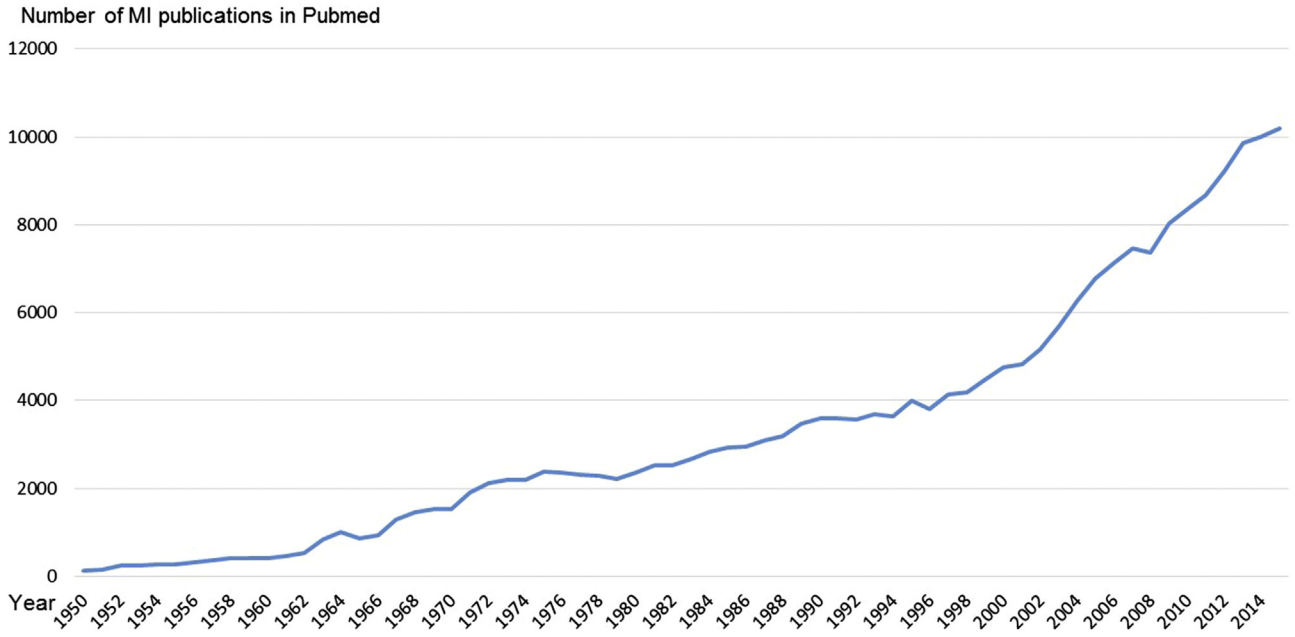


FIGURE 12.1 Number of publications on myocardial infarction on Pubmed.

Clinical practice based on the best available evidence improves the quality of healthcare and results in optimal outcomes for patients. Busy clinicians may be overwhelmed by the quantity of new research. For example, if a diligent cardiologist wanted to keep herself informed of the latest research on myocardial infarction in 1950, she would have to read 150 articles that year. By 2015, she would need to read well over 10,000 articles (see Fig. 12.1). Clearly, this is an impossible task. The conclusions drawn from an individual study may often contradict those resulting from another, oftentimes, without an obvious explanation for the discrepancy. When faced with making a specific clinical decision for a particular patient, the suitability of using the available evidence to inform that decision may not be straightforward to judge.

Developing guidelines helps inform clinical practice by making recommendations based on the totality of the most up-to-date evidence. By addressing a series of clinically relevant questions, the research is translated into information that informs specific clinical decisions. Directing the optimal use of healthcare resources (e.g., labor, diagnostics, hospital beds, drugs) through the use of guidelines improves efficiency. Efficiency, in this sense, means the use of limited healthcare resources in a way that would result in the maximum production of health, however, defined. By standardizing care across patients in different care settings and hospitals across the health system, the guidelines also ensure equity, here defined as equivalent care for equivalent patients.

In addition to the usual arguments underpinning the development of guidelines—quality, efficiency, and equity—it has been argued that "*the spread of guidelines represents a new regulation of medical care … Although the regulation of quality has traditionally been based on the standardization of professional credentials, since the 1960s it has intensified and been supplemented by efforts to standardize the use of medical procedures. The proliferation of collectively produced guidelines since the 1980s represents a growing effort to bring order and coherence to a rapidly expanding and heterogeneous medical domain*."[3]

Guidelines are an aid to clinical decision-making and not intended to infringe on the discretion of the clinician in treating an individual patient. By their nature, guidelines are summaries of the evidence that will be applicable to most patients but will not cover every patient or clinical situation that a clinician may deal with. Therefore, clinicians are expected to rely on their own judgment when using guidelines to inform decisions made for an individual patient. However, it would be expected that there is a clear justification when the management of a patient deviates from best practice as laid out in a guideline. Guidelines are usually produced by professional bodies, governmental agencies, national, nongovernment agencies, and academic institutions within particular national jurisdiction, though transnational cooperation on guideline production is common.

OBJECTIVE 12.2. SET THE FRAMEWORK FOR THE DEVELOPMENT OF A CLINICAL GUIDELINE

Before Developing a Guideline

Before deciding to initiate the development of a clinical guideline, it would be sensible to assess the need for it. This involves the specification of a clinical problem or a health system issue that can be addressed by a guideline. An objective search of important databases and key clinical evidence sources for relevant existing guidelines should be performed to ensure that no currently available guideline meets the requirements. The involvement of experts in assessing the suitability of an existing guideline to inform clinical practice will allow a judgment to be formed on whether any guidelines identified are relevant to the context to which they will be applied. To aid the process, the quality of existing guidelines can be assessed using an instrument such as AGREE.[4] Guidelines developed in closely related health system contexts, for patients that are similar, will usually be transferable, possibly with some adaptation. This is a judgment that has to be made on a case-by-case basis. When guidelines are adapted, a clear description of guideline adaptation methods is recommended.[5]

The AGREE instrument is a tool that assesses the methodological rigour and transparency in which a guideline is developed. It is a framework to:

1. assess the quality of guidelines;

2. provide a methodological strategy for the development of guidelines; and

3. inform what information and how information ought to be reported in guidelines

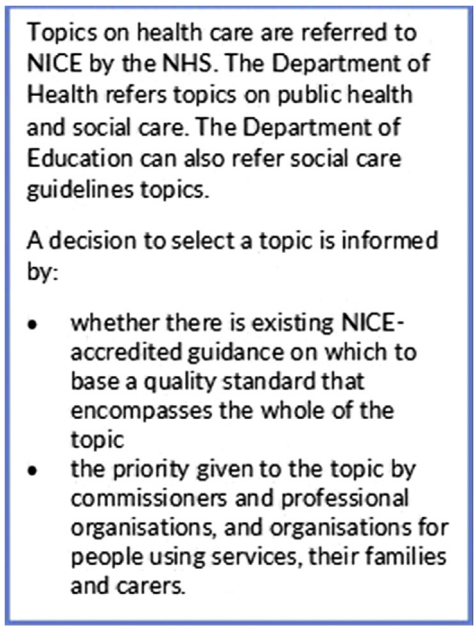
The users of the future guidance should be identified in advance to ensure that they are closely involved in the development process. This ensures that there is a "buy-in" to the final product by people who will be expected to implement the guideline or whose care will be impacted by the recommendations. Equally important, the people who will implement the guideline will also possess the expertise that is required to interpret the evidence and use clinical judgment in developing the guideline, whether from the perspective of a clinician, patient, or health system administrator. Lastly, before starting, it would be wise to consider in advance, the methods and channels that will be used to disseminate the guideline, with some early consideration given to the monitoring of uptake, as well as any training that will be required for implementation by end users. In addition, before commencing the guideline development, a project plan should be put together that specifies timelines and project costs.

Prioritization

It is usually not possible, due to resource constraints, or necessary to create a guideline for every condition. Efforts should be directed to developing guidelines which can have the greatest impact. The process and criteria for selecting and prioritizing topics should be transparent. Some considerations for the prioritization of topics for guideline development include the following:

- High prevalence disease or high use medical intervention
- High associated cost

- Effects on premature mortality and avoidable morbidity
- Evidence that medical care can make a difference in outcomes
- Knowledge of current variations in practice or that practice does not match some known practice parameters
- No existing guidelines of good quality to inform practice
- An expectation that a recommendation, when implemented, will improve health outcomes, reduce inequities, or unnecessary costs
- Implementation of a guideline will not exhaustively use available resources, and barriers to change are not likely to be so high that they cannot be overcomed.[6,7]


Topics on health care are referred to NICE by the NHS. The Department of Health refers topics on public health and social care. The Department of Education can also refer social care guidelines topics.

A decision to select a topic is informed by:

- whether there is existing NICE-accredited guidance on which to base a quality standard that encompasses the whole of the topic
- the priority given to the topic by commissioners and professional organisations, and organisations for people using services, their families and carers.

To avoid the selection of topics being influenced by stakeholders with a particular interest, the establishment of a process to agree on priorities will be required. The process should specify in advance who will take responsibility for the selection of guideline topics (e.g., the guideline producing organization, policy-makers, health authorities). In practice, it would be sensible to ensure that all relevant stakeholders are involved in the decision and agree with proceeding to guideline development. Criteria for establishing priorities should be applied using a systematic and transparent process, which includes consultation with stakeholders.

Who Is Involved?

A guideline development group should consist of a diverse range of stakeholders who are relevant to the topic. In particular, it is important to not only involve professionals with expertise in the topic but also the end users of guidance who will be responsible for its implementation, as well as patients on whom the guidance will impact. Individuals and groups involved in guideline development should have the range of technical skills that will be required, such as information retrieval, systematic reviewing, health economics, group facilitation, project management, writing, and editing. Careful consideration should be given to the appointment of the Chair of the committee who is responsible for leading and facilitating collaboration between group members and ensuring participation of less vocal members while avoiding imposing personal opinions. As group members will come from a variety of professional and nonprofessional backgrounds, training to understand the technical aspects of guideline development and support should be offered to optimize their ability to participate. Each participant should have a clear idea of the timelines and tasks for their participation, with detailed instructions on the activities, roles, and responsibilities of the group.

At NICE, stakeholders for guidelines include:

- national organisations for people who use health and social care services, their families and carers, and the public
- local Healthwatch organisations
- national organisations that represent health and social care practitioners and other people whose practice may be affected by the guideline, or who can influence uptake of the guideline recommendations
- public sector providers and commissioners of care or services
- private, voluntary sector and other independent providers of care or services
- companies that manufacture drugs, devices, equipment or adaptations, and commercial industries relevant to public health; organisations that fund or carry out research; government departments and national statutory agencies.

It is critical to consider in advance how patients will be involved in the development of guidelines, as the impact of the recommendations will be most keenly felt by them. Involvement of patients can consist of the following:

- incorporating individual patients or patient advocates in guideline development groups;
- a "one off" meeting with patients;
- a series of workshops with patients;
- eliciting the perspective of patients and the public as part of a larger stakeholder input exercise.[7]

Managing Conflict of Interests

Once group members are nominated, any potential conflicts of interests should be identified and addressed. Conflicts of interests of individual group members can cast doubt on the recommendations in the guideline if it is believed that these were the results of interests of, and pressures on, group members, rather than impartially arrived at through the consideration of evidence. Each member of a guideline development group should make a full declaration of competing interests, which should be recorded and addressed. The disclosure should include financial and nonfinancial conflicts of interest, and the methods for resolution of conflicts should be made explicit. A conflict of interest may require the recusal of a member from specific decisions that bring the conflict into operation.

Conflicts of interest can be defined as

- A set of circumstances that creates a risk that professional judgment or actions regarding a primary interest will be unduly influenced by a secondary interest
- "A divergence between an individual's private interests and his or her professional obligations such that an independent observer might reasonably question whether the individual's professional actions or decisions are motivated by personal gain, such as financial, academic advancement, clinical revenue streams or community standing"
- "A financial or intellectual relationship that may impact an individual's ability to approach a scientific question with an open mind"

In addition to the individual guideline group members, it is also essential to ensure that the views of the funding body have not influenced the content of the guideline. A guideline should disclose financial support for the development of both the evidence review and the guideline itself.

Key principles for developing guidelines

NICE develops guidelines according to the following core principles:

- Guidance is based on the best available evidence of what works, and what it costs.
- Guidance is developed by independent and unbiased Committees of experts.
- All our Committees include at least 2 lay members (people with personal experience of using health or care services, or from a community affected by the guideline).
- Regular consultation allows organisations and individuals to comment on our recommendations.
- Once published, all NICE guidance is regularly checked, and updated in light of new evidence if necessary.
- We are committed to advancing equality of opportunity and ensuring that the social value judgements we make reflect the values of society.
- We ensure that our processes, methods and policies remain up-to-date.

OBJECTIVE 12.3. INTRODUCE THE METHODOLOGY OF CLINICAL GUIDELINE DEVELOPMENT

Developing a Guideline

The development of a guideline can be broken down into a series of tasks. Guideline development from different organizations may follow a different set of tasks chosen from an exhaustive menu.[5] The individual steps involved in guideline production and a comparison of international guideline development programs vary, but they usually include a core set of activities that can be considered essential to guideline development.[8] A clear process that specifies which of the tasks are included in guideline production should be decided in advance. The guideline development process at National Institute for Health and Care Excellence (NICE) is presented in Fig. 12.2.

Definition and Purpose of Scope

Before embarking on the guideline development, the overall objective(s) of the guideline should be described. This will include the specific health questions the guideline will cover, as well as the population (e.g., patients, public) to whom the guideline is meant to apply. The framework should describe the epidemiology of the disease or condition, the aspects of care, and the settings to be covered by the guideline.[9] The scope of a guideline may include primary or secondary care, prevention, diagnosis, and management. The scope sets the boundaries for what the guideline will make recommendations

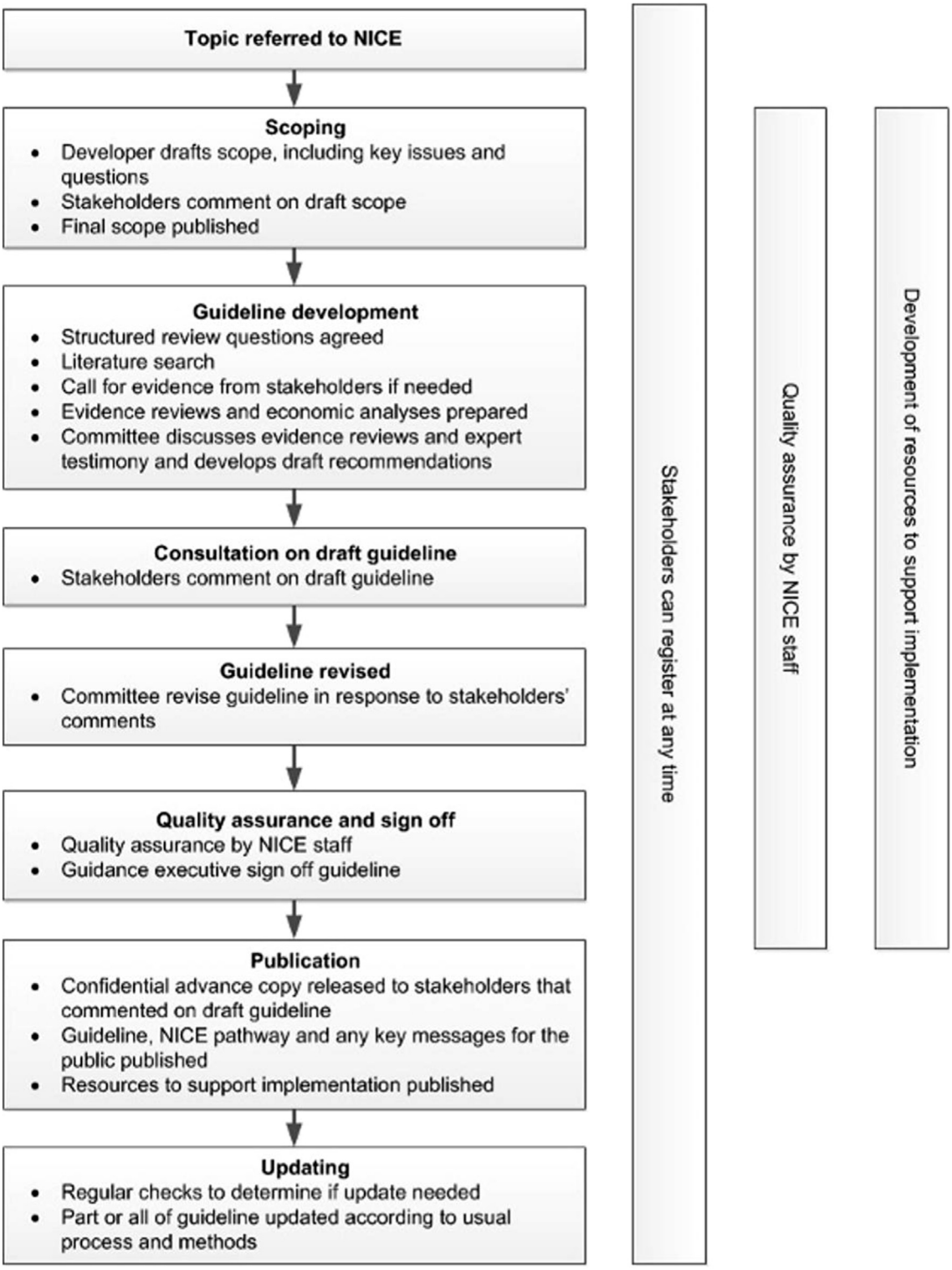


FIGURE 12.2 The guideline development process at NICE.

on and acts as a guide to subsequent steps that are part of the development process. It will also set the limits on the recommendations that will form the output of the development process. Developing the scope will require its own subprocess, which will involve drafting and discussing a scope, consulting on it, and finalizing the scope based on comments received during consultation. Scoping is an important process, as it will determine the boundaries of the guideline, including relevant clinical questions, evidence search, and recommendations.

Purpose of the scope

The scope sets boundaries that ensure the work stays within the referral. The scope:

- defines the population(s) and setting(s) that will and will not be covered
- describes what the guideline will consider
- identifies the key issues and lists the key questions that will be considered
- describes the economic perspective(s) to be used.

The scope will usually include:

- a brief description of the guideline topic
- a brief overview of the context (current policy and practice) in which the guideline will be developed
- a summary of why the guideline is needed and where it will add value
- how the guideline will link to other NICE recommendations and quality standards
- potential equality issues
- health inequalities associated with socioeconomic factors and with inequities in access for certain groups to healthcare and social care, and opportunities to improve health.

Gathering Evidence

Guideline development involves both a technical process (i.e., systematic reviews of relevant evidence) and a social process (i.e., interpretation of the results of the systematic review and development of recommendations).[7]

Developing Clinical Questions

The first step is to develop a series of clinical questions on which guidance will be produced. Within a large disease area, specific questions that relate to clinical practice help focus the search for evidence and provide guidance. For example, the NICE clinical guideline on hypertension in adults asks a series of questions on the management of hypertension such as, "*In adults with primary hypertension, which is the most clinically and cost-effective anti-hypertensive monotherapy (ACEi vs ARB) for first-line treatment, and does this vary with age and ethnicity?*," and, "*In adults with primary hypertension, which is the most clinically and cost-effective combination of anti-hypertensives (A + C or A + D) for second-line treatment, and does this vary with age and ethnicity?*"[10]

At NICE, for each question, the framework is defined using PICO.

The PICO framework

Population: Which population should the guideline cover? How can it be best described? Are there subgroups that need to be considered?

Intervention: Which intervention, treatment or approach should be used?

Comparators: Are there alternative(s) to the intervention being considered in established clinical practice?

Outcome: Which outcomes should be considered to assess how well the intervention is working? What is really important for people using services?

Systematic Search for Evidence

Once the review questions are developed, a systematic strategy is used to search for evidence. Systematic searches specify the parameters of the evidence that is being sought, for example, based on the PICO table, but also in terms of the types of study designs, specifying inclusion and exclusion criteria or any other limitations that are relevant. They form the basis of an electronic search of important bibliographic databases using sensitive keywords. Systematic methods must be clearly documented so that they can be rerun by other investigators and the results reproduced. A systematic review will require specialized skills in determining search terms, conducting the search in databases, filtering the results of the search according to predefined criteria, and summarizing and presenting the outcomes.

Systematic review

A review that summarises the evidence on a clearly formulated review question according to a predefined protocol, using systematic and explicit methods to identify, select and appraise relevant studies, and to extract, analyse, collate and report their findings.

Meta-analysis

The use of statistical techniques in a systematic review to integrate the results of several studies.

For more details on systematic review and meta-analysis, refer to the Cochrane handbook.

Appraising Identified Research Evidence

Once relevant publications are identified using the search strategy, they need to be appraised for quality and relevance using objective instruments. This allows to clearly describe the strengths and limitations of the body of evidence. NICE has adopted the GRADE approach.[11] This is a validated assessment tool to evaluate the quality of evidence underpinning

guideline recommendations. It is used to determine the strength of the recommendations in the guideline. Excluding or including studies that are deemed to be of poor quality, and then looking at the impact on the overall decisions, serves as a sensitivity analysis on the robustness of the conclusions drawn from the evidence and the recommendations.

The GRADE approach to assessing the quality of evidence for intervention studies

- study limitations (risk of bias) – the internal validity of the evidence
- inconsistency – the heterogeneity or variability in the estimates of treatment effect across studies
- indirectness – the degree of differences between the population, intervention, comparator for the intervention and outcome of interest across studies
- imprecision (random error) – the extent to which confidence in the effect estimate is adequate to support a particular decision
- publication bias – the degree of selective publication of studies.

From Evidence to Recommendations

Group decision-making essentially involves three phases: orientation (defining the problem), evaluation (discussion of decision alternatives), and control (deciding which of the alternatives is to prevail).[7]

Ideally, the guidelines group will reach its recommendations through consensus. However, as this situation may not materialize in every case, alternate methods to reach an agreement must be planned for. One option is the use of objective consensus development techniques such as the Delphi method, Nominal Group technique, or more simply as at NICE, by majority voting. Such methods should be clearly established and communicated to group members before commencing on developing recommendations. A savvy Chair of the group, by facilitating decisions and reducing areas of divergence, is an essential ingredient for reaching consensual decisions and avoiding conflict.

When making recommendations, the group will need to be mindful of the future resource implications for implementation of the proposed recommendations. This may be done through the means of explicit incorporation of considerations of cost-effectiveness, as well as affordability and resource implications in the guideline development process. In this case, specific recommendations within the guideline relating to interventions are subject to additional cost-effectiveness analysis before a decision is made to include a specific recommendation in the guideline. At NICE, the need for an economic evaluation depends on the potential overall expected benefit and resource implications of intervention, both for individual people and the population as a whole, and the degree of uncertainty in the economic evidence review and the likelihood that economic analysis will clarify matters. The economic evaluation may not be warranted when:

- clinical efficacy of an intervention is not established
- it is clear that either one of the benefits or costs of the intervention outweighs the other
- when existing cost-effectiveness analysis is suitable for decision-making
- when the costs, benefits, or budget impact is small
- when the evidence or methodology is not suitable for the conduct of an analysis.

When conducting a cost-effectiveness analysis, a clear reference case will be required as a framework to ensure consistency between decisions. At NICE, the reference case used is the same as that specified for appraisals of individual

health technologies. In addition, recommendations made through other programs at NICE, based on the assessment of cost-effectiveness, will be incorporated into guidelines to maintain consistency between recommendations across the Institute.

Recommendations in a guideline do not operate within a value-free environment. Values, the relative importance or consequences of a decision (e.g., outcomes relating to benefits, harms, burden, and costs), often influence every recommendation in ways that may not be transparent. Ethical considerations, concepts that determine what is right, also play a role.[12] It is, therefore, useful to integrate such values into guideline recommendations in an explicit fashion. The values used in making recommendations should reflect those of the people affected and whose care will be impacted by the guideline. Judgments should be explicit and should be informed by input from those affected (e.g., citizens, patients, clinicians, policy-makers).

The guideline development group uses the expertise of its members to interpret the available evidence to make recommendations that are actionable by clinicians within the health service. The group needs to take into consideration the health benefits, harms, risks, costs, and values in making recommendations. The wording of the recommendation should make explicit the link to the evidence underpinning it. Recommendations should be worded in clear and unambiguous language as to what action is to be undertaken, who undertakes it, and in what circumstances. Where there is a set of options for management of a condition, these should be laid out as options, with an order of preference, if one exists.

For each recommendation, the guideline should use a rating system that comments on the quality and reliability of the evidence, as well as the strength of the recommendation. The grading of evidence for each recommendation can be based on any validated score for classifying the strength of evidence. Lastly, a series of research recommendations identifying gaps in the available evidence to inform the guideline, which can act as a spur to guide to further research, should be included as part of the document. The research focused on evidence gaps can help inform any further updates or iterations of the guideline. This approach can help plug gaps in knowledge relating to the optimal management of patients and use of interventions and healthcare resources.

Examples of standardised terms for describing the strength of the evidence

No evidence 'No evidence was found from English-language trials published since 1990...'

Weak evidence 'There was weak evidence from 1 (−) RCT'.

Moderate evidence 'There was moderate evidence from 2 (+) controlled before and after studies'.

Strong evidence 'There was strong evidence from 2 (++) controlled before and after studies and 1 (+) RCT'.

Inconsistent evidence. Further commentary may be needed on the variability of findings in different studies. For example, when the results of (++) or (+) quality studies do not agree.

Consultations

Following the development of draft guidelines, these should be consulted on with all stakeholders. In addition, at NICE, a public consultation is undertaken. NICE guidelines are also subject to an external peer review by experts, much as would be expected with any scientific publication. Following consultation, the guideline group should meet to consider the comments received and make any amendments to the guideline as it sees fit. The guideline recommendations are then finalized for publication. At NICE, in an additional step, the guideline is signed-off by senior management before release. This acts as a final check on ensuring that guidelines are in keeping with the Institute's wider remit and the need to meet legal obligations, as well as to help identify controversial recommendations that might elicit a negative response once the public and clinical community are made aware.

Publishing

To facilitate adoption, a guideline should be disseminated widely through multiple channels. Although the use of the Internet would nowadays be considered essential, some regions may require a printed version of the guidance. At NICE, the guideline in its entirety, including the evidence underpinning it, is available in digital format on the NICE website. Electronic documents also include intermediate steps of development that allow the public to follow the various stages of development. Guidelines may need to be simplified and published in a format that is accessible to the lay audience. Production of a shorter version of the guideline limited to key recommendations and actions should be considered.

OBJECTIVE 12.4. EXPLAIN THE PROCESS OF IMPLEMENTATION AND UPDATE OF A CLINICAL GUIDELINE

Implementation

The release of a guideline does not automatically guarantee that its recommendations are put into practice. Firstly, there needs to be a plan for the guideline to be advertised and for people who are working within the health service to be made aware of its existence. The guideline should also provide advice and tools on how the recommendations can be put into practice, with some consideration given to barriers to implementation and suggestions for how these can be overcome. Resources that will aid in the implementation of a guideline include educational tools such as flowcharts and algorithms. A significant barrier to implementation is usually the resource impact of the recommendations. Even when these show cost savings in the long term, implementation of recommendations usually carries an immediate cost, such as when new services need to be developed and new people employed. The use of budget impact and budgeting tools may help managers adequately plan for the financial impact of guideline recommendations.

When releasing a guideline, planning for the monitoring of uptake and impact of the guideline for patient and system outcomes should be incorporated. The development and release of clinical audit and evaluation criteria to accompany the guideline could aid in the monitoring of guideline implementation. Data collection tools, such as audit templates, could accompany the guideline. Separately, additional monitoring for uptake, implementation, and impact of guidelines will help justify the existence of a guideline program. The feedback into the effectiveness of the existing program will help implement necessary changes to the guideline development processes and improve impact.

Reviewing and Updating Guidelines

A guideline is based on evidence collected that is retrospective at the point of the search. Almost from the point of its publication, the evidence on which it is based is out-of-date, as the relentless accumulation of new evidence proceeds. Therefore, a guideline development program should include a process for the review and update of guidelines. Each guideline should have an expiration date, at which point a clearly defined process will determine the requirement for updating recommendations based on new evidence that has become available subsequent to the guideline publication. A guideline may need to be updated as the evidence on the benefits and harms of interventions changes, new interventions become available, or changes occur in the available healthcare resources, health system priorities, or outcomes and values placed on outcomes.

At NICE, the need for updating a guideline is reviewed every 2 years. At 2, 6, and 10 years after a guideline is published, the review is limited to the defined scope, and a search is conducted to identify new evidence. At 4 and 8 years after guideline publication, the areas covered by the guideline, but not within the scope, are also addressed. Following a review, a decision may be taken not to update a guideline based on the lack of new evidence, with a plan to review the evidence at a further time point. When no new evidence is anticipated in the future, the guideline may be moved to a static list, where no further reviews of new evidence or updates are anticipated. However, it is more likely that the guideline will need to be updated. This can be a full update within the scope of the original guideline or a new scope can be developed highlighting new relevant areas which were not within the remit of the original scope. A new scope may also be developed where certain areas are no longer relevant or prioritized. Further outcomes of a review may be a partial update of the guideline, limited to defined sections or recommendations, when the new evidence identified will not require amendments to the whole guideline. A partial update may require a new scope or it can be based on the original scope. Lastly, a guideline may be "refreshed" by adding amendments to the wording of recommendations reflecting the current practice context and, sometimes, to meet current editorial standards.

Key elements of the process for checking whether a published guideline needs updating	
Time since publication	**Key elements of the process**
2 years 6 years 10 years	• Limited to scope of published guideline • Limited evidence review and summary of new evidence • Intelligence gathering from questionnaires, external enquiries, related NICE guidance, updated national policy, medicines licensing information and information on implementation • No consultation
4 years 8 years	• Also considers key areas within the referral of the guideline, but outside the scope of the published guideline • Literature search and summary of new evidence • Intelligence gathering from questionnaires, external enquiries, related NICE guidance, updated national policy, medicines licensing information and information on implementation • Consultation only when 'no update' considered

CONCLUSION

Clinical guidelines are widely used as a means of improving the quality of healthcare delivery. Having a wide range of stakeholders participating in guideline development will improve their relevance and uptake. In particular, care must be taken to include patients in guideline development groups. The first step in creating a guideline is to develop a scope and define specific clinical questions that will be addressed. Guidelines should be informed by a systematic search for evidence. Guideline development groups should have clear processes for reaching consensus on recommendations. Recommendations should be graded according to the strength of the evidence underpinning them. A wide consultation on draft recommendations should be conducted before publication. Guidelines should be accompanied by tools that help end users implement recommendations. Regular reviews and updates of the guideline will ensure that they remain relevant and useful.